Kansas Department of Health and Environment

Domestic and International Infectious Disease Report

March 18, 2026

The purpose of this report is to provide healthcare providers and healthcare facilities with a summary of activity for priority domestic and international infectious diseases. Visitors are expected to arrive for the 2026 FIFA World Cup as early as March 2026. Please use this information when evaluating a patient's recent travel history and symptom presentation. For additional information, please visit: https://www.kdhe.ks.gov/travelhealth.

Domestic (Within the United States): Last 30 Days

Medium Priority: Seasonal Influenza in the United States

(Updated from Previous Week)

Overall seasonal influenza activity remains elevated nationally but is decreasing in most areas of the country. Influenza A activity continues to decrease while trends in influenza B activity vary by region. According to the US Centers for Disease Control and Prevention, seasonal influenza accounted for 2.4% of all emergency department visits for the week ending March 7, 2026, a decrease from the previous week. National hospitalization data for influenza decreased slightly from 3.4 admissions per 100,000 population to 3.0 admissions per 100,000 for the week ending February 14, 2026. Influenza-related mortality decreased from the previous week with 0.5% of all deaths being attributed to seasonal influenza for the week ending March 7, 2026. The number of influenza-related pediatric deaths increased by 11 for a total of 101 deaths this season.

Medium Priority: COVID-19 in the United States

(Updated from Previous Week)

COVID-19 activity is decreasing nationally but is elevated in some areas of the country. According to the US Centers for Disease Control and Prevention, COVID-19 accounted for 0.5% of all emergency department visits for the week ending March 7, 2026, a stable trend from the previous week. National hospitalization data for COVID-19 increased very slightly from 1.3 admissions per 100,000 population to 1.4 admissions per 100,000 population for the week ending February 14, 2026. COVID-related mortality data was corrected down to 0.5% of all deaths being attributed to COVID-19 for the week ending March 7, 2026.

Medium Priority: Respiratory Syncytial Virus in the United States

(Updated from Previous Week)

RSV activity is elevated and increasing in some regions of the country. According to the US Centers for Disease Control and Prevention, the typical RSV season in the United States runs from October through March, with activity usually peaking between December and February. According to the US Centers for Disease Control and Prevention, RSV accounted for 0.5% of all emergency department visits for the week ending March 7, 2026, a very slight decrease from the previous week. Data from the Respiratory Syncytial Virus Hospitalization Surveillance Network (RSV-NET) show that the weekly rate of RSV associated hospitalizations increased slightly from 2.5 admissions per 100,000 population for the week ending February 7, 2026, to 2.7 admissions per 100,000 population for the week ending February 14, 2026. Compared to the previous six seasons, typically admissions start to level off or decrease at this point in the season. This may indicate that RSV activity could extend beyond March.

Medium Priority: Measles in Multiple States

(Updated from Previous Week)

As of March 12, 2026, the US Centers for Disease Control and Prevention reported 1,362 confirmed measles cases in the United States in 2026. Among these, 1,353 measles cases were reported by 31 jurisdictions: Alaska, Arizona, California, Colorado, Florida, Georgia, Idaho, Illinois, Kentucky, Maine, Massachusetts, Minnesota, Missouri, Nebraska, New Mexico, New York City, New York State, North Carolina, North Dakota, Ohio, Oklahoma, Oregon, Pennsylvania, South Carolina, South Dakota, Texas, Utah, Vermont, Virginia, Washington, and Wisconsin. A total of 9 measles cases were reported among international visitors to the United States.

As of March 16, 2026, the Colorado Department of Public Health and Environment reported 11 measles cases since February from four counties: Adams (5), Broomfield (4), Weld (1), and Arapahoe (1). As of March 18, 2026, the North Dakota Health and Human Services reported two additional cases bringing the total to 26 cases during 2026; four hospitalizations have been reported.

As of March 17, 2026, the South Carolina Department of Public Health reporting 997 cases of measles centered around Spartanburg County. While most cases are known close contacts to known cases, South Carolina reported several known public exposure sites. As of the same date, the North Carolina Division of Public Health reported 24 cases since December 2025; no new cases have been reported since the week ending February 21, 2026.

As of March 17, 2026, the Utah Department of Health and Human Services reported 443 measles cases driven by an outbreak in the southwest part of the state; 98 measles cases were reported to public health in the last 3 weeks. As of the same date, the Arizona Department of Health Services reported two new cases in March. A total of 278 cases has been associated with the outbreak along the Arizona–Utah border; 281 cases have been reported in Arizona since 2025. As of March 6, 2026, Washington State Department of Health reported 26 measles cases; 21 cases are outbreak-associated. The Florida Department of Health reported 98 confirmed measles cases across Collier County from January 1 to March 7, 2026; this county is home to Ave Maria University which is experiencing an outbreak.

Medium Priority: Sexually Transmitted Fungal Infection Outbreak in the United States

(No Update, Still Relevant)

As reported in the February 18 issue of this report, on February 11, 2026, the Minnesota Department of Health issued a Health Advisory advising health care providers about an outbreak of *Trichophyton mentagrophytes* genotype VII (TMVII). Two cases were also identified in San Francisco in March and December 2025 in persons with no known exposure, suggesting local transmission.

International (Canada, Mexico, All FIFA WC Qualifying Countries): Last 30 Days

Algeria, Argentina, Australia, Austria, Belgium, Brazil, Canada, Cabo Verde (Cape Verde), Colombia, Croatia, Curacao, Ecuador, Egypt, England, France, Germany, Ghana, Haiti, Iran, Ivory Coast (Côte d'Ivoire), Japan, Jordan, Mexico, Morocco, New Zealand, Norway, Panama, Paraguay, Portugal, Qatar, Saudi Arabia, Scotland, Senegal, South Africa, South Korea, Spain, Switzerland, The Netherlands, Tunisia, Uruguay, and Uzbekistan

High Priority: Mpox in Brazil

(Updated from Previous Week)

As of March 17, 2026, Brazil has reported 149 confirmed mpox cases and 539 suspected cases, predominantly in the Southeast region. Ceará has recorded its second case this year, leading to increased surveillance due to eight suspected cases. The newly identified Clade Ib variant, associated with heightened transmissibility and severity, has been confirmed in Argentina, with two cases also detected in Brazil, raising alarms across the Americas. Espírito Santo has reported its third case in 2026, with a total of 50 notifications, including three confirmed and two cases under investigation. A confirmed imported case in Acre has triggered contact tracing efforts involving health authorities in São Paulo and Bolivia, with no local transmission reported so far. The situation highlights the urgent need for ongoing monitoring and preventive measures to control the spread of the virus.

High Priority: Mpox in Europe

(Updated from Previous Week)

As reported by the European Centre for Disease Prevention and Control, since February 1, 2026, and as of March 12, 2026, 230 mpox cases have been reported from 14 European Union/European Economic Area (EU/EEA) countries: Spain (57), Germany (49), France (31), Italy (27), Ireland (12), Netherlands (12), Belgium (10), Portugal (10), Sweden (9), Austria (7), Czechia (3), Greece (1), Norway (1) and Poland (1).

Since the start of the mpox outbreak and as of March 12, 2026, 26,398 confirmed cases of mpox have been reported from 29 EU/EEA countries: Spain (9,482), Germany (4,682), France (4,476), Netherlands (1,619), Portugal (1,364), Italy (1,358), Belgium (936), Austria (394), Sweden (383), Ireland (342), Poland (258), Denmark (247), Greece (162), Norway (137), Czechia (115), Hungary (90), Luxembourg (66), Romania (51), Malta (49), Slovenia (48), Croatia (39), Finland (21), Slovakia (19), Iceland (18), Bulgaria (11), Estonia (11), Cyprus (6), Latvia (6) and Lithuania (6). Deaths have been reported from: Spain (4), Belgium (2), Portugal (2), Austria (1) and Czechia (1). Within the same period (since the beginning of the

outbreak), the following Western Balkan countries have reported confirmed cases of mpox: Serbia (40), Bosnia and Herzegovina (9), Montenegro (2), North Macedonia (2), Albania (1) and Kosovo (1). In addition, 76 cases have been reported from Turkey, one of which was clade Ia and five clade Ib, the most recent being reported in December 2025.

High Priority: Mpox in Africa

(No Update, Still Relevant)

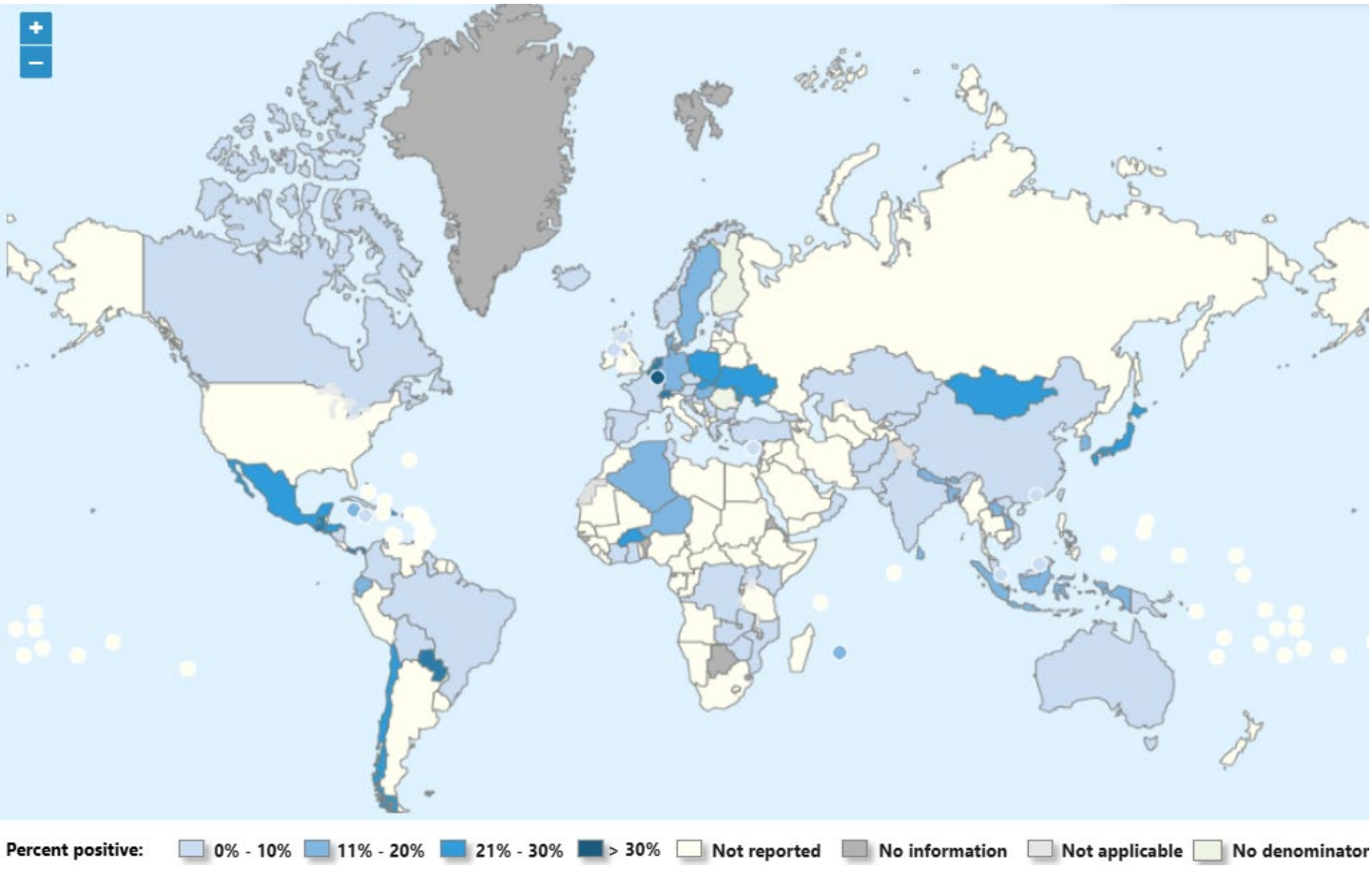
According to the World Health Organization, as of March 1, 2026, there have been 888 laboratory confirmed cases of mpox reported in Africa within the last six weeks. During this period, Madagascar (357) and the Democratic Republic of the Congo (216) reported the highest case counts. During the same time period, Uganda (1), Kenya (2), Madagascar (1), the United Republic of Tanzania (1) also reported mpox-related deaths.

The US Centers for Disease Control and Prevention has issued a Level 2 – Practice Enhanced Precautions Travel Health Notice regarding clade II mpox in Ghana and Liberia.

Medium Priority: Influenza in Multiple Countries

(Updated from Previous Week)

The World Health Organization is reporting that, globally, influenza detections continued to decline, and influenza A viruses were predominant among influenza detections with a slight increase in the proportion of influenza B virus detections. For the week ending March 8, 2026, the following countries experienced greater than 30% positivity: Panama, Paraguay, Belgium, and the Netherlands.



Source: World Health Organization. Integrated influenza and other respiratory viruses surveillance output, Week 10.

Medium Priority: Respiratory Syncytial Virus in Tunisia

(Updated from Previous Week)

The World Health Organization is reporting that, globally, RSV positivity remains low. However, for the week ending March 8, 2026, Tunisia reported 38.8% positivity, a decrease from the previous week.

Medium Priority: SARS-CoV-2 in Europe

(Updated from Previous Week)

The World Health Organization is reporting that, globally, SARS-CoV-2 (COVID-19) activity remains low. However, for the week ending March 8, 2026, the Netherlands reported 78.6% positivity.

Medium Priority: Chikungunya in Europe

(Updated from Previous Week)

On March 10, 2026, the US Centers for Disease Control and Prevention issued a Level 2 – Practice Enhanced Precautions Travel Health Notice regarding chikungunya for Mayotte. Mayotte is an overseas department of France off the coast of Southeastern Africa where chikungunya activity has seen a resurgence in recent years.

According to the European Centre for Disease Prevention and Control, travel-associated cases continue to be reported from Seychelles. The US Centers for Disease Control and Prevention issued a Level 2 –

Practice Enhanced Precautions Travel Health Notice regarding chikungunya in Seychelles on February 10, 2026.

Medium Priority: Chikungunya in the Americas

(Updated from Previous Week)

According to data from the Pan American Health Organization (PAHO), Colombia last reported 11 chikunguyna cases for the week ending February 7, 2026. Uruguay last reported 0 cases for the week ending February 14, 2026; the last reported case was on July 26, 2025. For the week ending February 28, 2026, Panama reported 0 cases; the last case was reported on January 3, 2026. For the week ending March 7, 2026, Mexico reported 12 cases; the last case was reported on February 21, 2026. For the same week, Argentina reported 1,647 cases and Brazil reported 20,338 cases; each reported their last case on the same day. For the same week, Paraguay reported 3 cases; the last case was reported in Paraguay on February 28, 2026.

The US Centers for Disease Control and Prevention has issued Level 2 – Practice Enhanced Precautions Travel Health notices for Suriname and Bolivia, both located in South America, in response to recent Chikungunya outbreaks.

Medium Priority: Measles in Canada

(Updated from Previous Week)

According to data from the Public Health Agency of Canada (PHAC), as of February 28, 2026, there have been a total of 5,460 probable and confirmed measles cases reported in Canada during 2025, and 413 probable and confirmed measles cases reported during 2026. During 2026, cases have been reported by 7 jurisdictions: Manitoba (269), Alberta (103), British Columbia (18), Nova Scotia (10), Saskatchewan (5), Ontario (5), and Quebec (3).

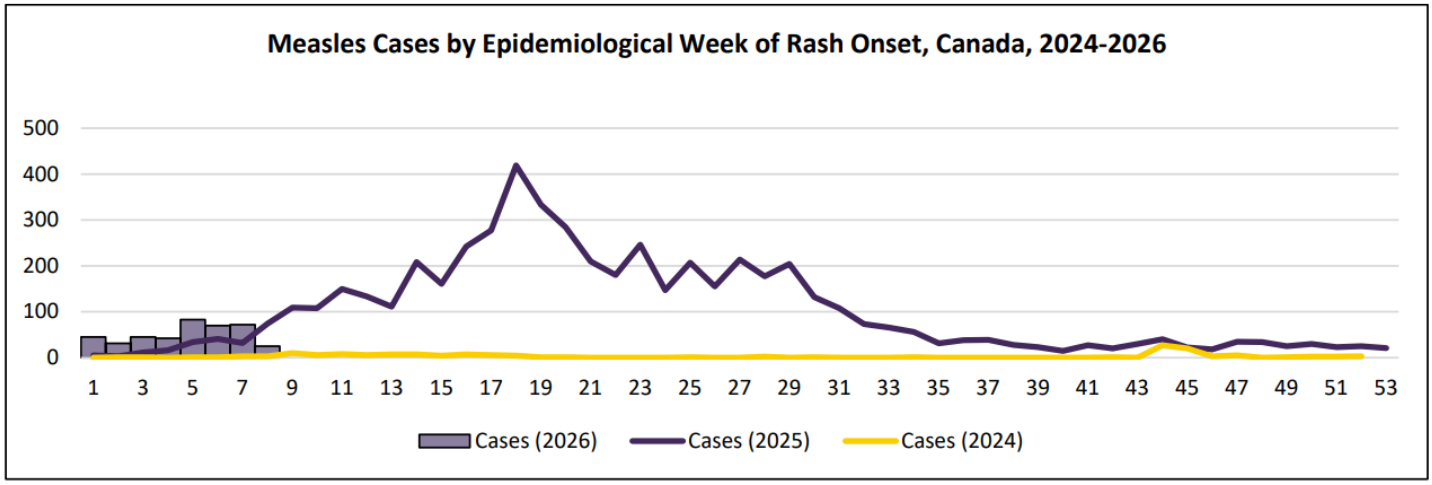


Figure Notes: Data as of February 28, 2026, and includes probable and confirmed cases.

Source: New York State Department of Health. Global Health Update. March 12, 2026.

Medium Priority: Measles in Mexico

(Updated from Previous Week)

According to data from the Secretary of Health of Mexico, as of March 11, 2026, there have been a total of 6,452 confirmed measles cases and 27 deaths reported in Mexico during 2025, and 6,680 confirmed cases and 7 deaths reported during 2026. Incident cases were primarily reported in Jalisco (528), Mexico City (82), and Chiapas (63). The number of confirmed cases reported during 2026 is now higher than the number of confirmed cases reported during 2025.

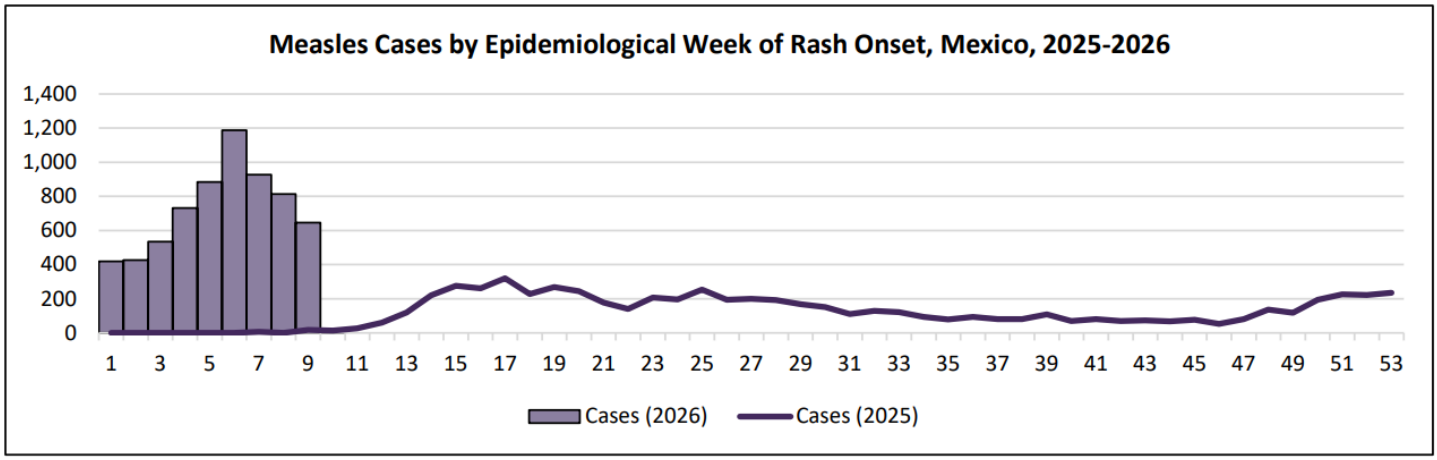


Figure Notes: Data as of March 4, 2026, and includes confirmed cases only (4 missing from figure).

Source: New York State Department of Health. Global Health Update. March 12, 2026.

Medium Priority: Yellow Fever in the Americas

(Updated from Previous Week)

According to data from the Pan American Health Organization (PAHO), as of March 13, 2026, there have been a total of 36 confirmed yellow fever cases and 15 deaths reported in the Americas during 2026. Of these cases, 27 were reported by Colombia, one by Bolivia, two cases by Peru, and six by Venezuela—the latter reporting cases for the first time in 2026. Most fatal cases were reported by Colombia (14/15) with one fatality reported by Bolivia.

Yellow fever remains endemic in tropical regions of Africa and South America with periodic outbreaks causing significant morbidity and mortality. Expansion into previously low-risk areas has been observed in recent years, reflecting shifting ecological and population exposure dynamics. Approximately 15% of infections progress to severe disease, with case fatality rates up to 50% among severe cases.

The US Centers for Disease Control and Prevention currently has posted a Level 2 – Practice Enhanced Precautions Travel Health notice regarding yellow fever in Colombia.

Medium Priority: Dengue in Colombia

(No Update, Still Relevant)

On January 22, 2026, the US Centers for Disease Control and Prevention updated their Level 1 – Practice Usual Precautions Travel Health Notice regarding global dengue and continues to recommend travelers to Colombia practice usual precautions to prevent dengue infection.

Medium Priority: Rocky Mountain Spotted Fever in Mexico

(No Update, Still Relevant)

The US Centers for Disease Control and Prevention issued a Level 1 – Practice Usual Precautions Travel Health Notice on December 23, 2025 for Rocky Mountain Spotted Fever in Mexico.

Medium Priority: Measles in Multiple Countries

(No Update, Still Relevant)

Measles cases are rising in many countries around the world. The affected countries include: Algeria, Argentina, Australia, Austria, Belgium, Brazil, Cape Verde, Colombia, Côte d'Ivoire (Ivory Coast), Croatia, Curaçao, Ecuador, Egypt, France, Germany, Ghana, Haiti, Iran, Japan, Jordan, Mexico, Morocco, New Zealand, Norway, Panama, Paraguay, Portugal, Qatar, Saudi Arabia, Senegal, South Africa, South Korea, Spain, Switzerland, The Netherlands, Tunisia, England, Uruguay and Uzbekistan.

Low Priority: Varicella in Uruguay

(No Update, Still Relevant)

As reported in the March 11, 2026, issue of this report, varicella cases are being reported across Uruguay, many in children and concentrated in Montevideo during a seasonal rise.

Low Priority: Diphtheria in South Africa

(Updated from Previous Week)

According to the latest Diphtheria Situational Report (Week 10), two new laboratory-confirmed cases of toxigenic diphtheria, were reported from the Western Cape; no new asymptomatic carriers of toxigenic *C. diphtheriae* were identified. Respiratory diphtheria is a vaccine-preventable illness caused by toxigenic *C. diphtheriae* (and more rarely *C. ulcerans* or *C. pseudotuberculosis*) and can occur in persons of all ages. Between January 1, 2024, and March 8, 2026, South Africa identified 96 confirmed respiratory diphtheria cases, two probable cases, and 62 asymptomatic carriers of toxigenic *C. diphtheriae* detected through contact tracing. The majority of confirmed cases and carriers (77%, 122/158) were from the Western Cape, comprising 72 respiratory diphtheria cases and 50 asymptomatic carriers. Undetected transmission chains in the community are suspected.

Low Priority: Global Polio

(Updated from Previous Week)

On March 9, 2026, the US Centers for Disease Control and Prevention updated its Level 2 – Practice Enhanced Precautions Travel Health notice regarding global polio. The countries affected include Afghanistan, Algeria, Angola, Benin, Burkina Faso, Cameroon, Central African Republic, Chad, Côte d'Ivoire, Democratic Republic of Congo, Djibouti, Ethiopia, Gaza, Germany, Guinea, Israel, Laos, Namibia, Niger, Nigeria, Pakistan, Papua New Guinea, Poland, Senegal, Somalia, South Sudan, Sudan, Tanzania, United Kingdom, and Yemen.

Watchlist: Hantavirus in Argentina

(New)

According to BEACON, Salta Province, Argentina, has reported 12 hantavirus cases and two deaths during January 4 to March 16, 2026. Most infections occurred in Orán Department (10 cases), including eight in San Ramón de la Nueva Orán and two in Colonia Santa Rosa. One additional case each was reported in General Güemes and Tartagal in General José de San Martín Department.

The reported cases indicate ongoing hantavirus transmission in northwestern Argentina, a region where ecological conditions support the circulation of rodent reservoirs. The concentration of cases in Orán Department suggests a localized exposure risk in environments where human activities intersect with rodent habitats, including rural settlements, agricultural areas, and border regions.

The detection of 12 confirmed cases within the first 10 epidemiological weeks of 2026 indicates early seasonal activity in a province historically affected by hantavirus infections.

Watchlist: Unknown Suspected Hemorrhagic Fever in the Democratic of the Congo

(Updated from Previous Week)

In follow-up to the unknown hemorrhagic illness cluster in Bokila village reported in the March 11, 2026, issue of this report, samples from the fatal case tested negative for Ebola virus according to local laboratory reports. Suspected cases with similar symptoms, including severe headache, vomiting, diarrhea, and hematemesis (vomiting blood), remain under investigation and the causative pathogen has not been identified.

Laboratory confirmation and clinical details have not been reported for the separate suspect Ebola case being investigated in Beni, North Kivu Province.

Watchlist: Venezuelan Hemorrhagic Fever in Venezuela

(No Update, Still Relevant)

As reported in the March 11 issue of this report, an outbreak of Venezuelan Hemorrhagic Fever (FHV) was reported in the state of Barinas, Venezuela. A local media source reported a roughly 50% rise in mortality from FHV in the first quarter of 2026 in five Barinas's 12 municipalities. The source reported 31 deaths throughout 2025 compared to at least 21 deaths during January and February 2026. This information cannot be officially confirmed due to media restrictions in the country. The current situation is complex since symptoms can overlap with the recent yellow fever outbreak, and health authorities have not made any clarification statement.

Watchlist: Human Influenza A (H1N1) Swine Variant in Spain

(No Update, Still Relevant)

As reported in the March 4 issue of this report, according to the Week 9 Communicable Disease Threat Report from the European Centre for Disease Prevention and Control, in February, authorities in Spain reported a confirmed human case of swine influenza A(H1N1)v; the case had no known history of exposure to pigs or a contaminated environment and the epidemiological investigations are ongoing.

Watchlist: Lassa Fever in Nigeria

(Updated from Previous Week)

According to the latest Lassa Fever Situational Report from the Nigeria Centre for Disease Control and Prevention (NCDC), there were 65 confirmed and 460 suspected cases reported between February 23 and March 1, 2026. Six new healthcare workers were affected during the reporting week. For the same week, there were 9 deaths reported. Cumulatively for 2026, 109 deaths have been reported with a Case Fatality Rate (CFR) of 23.2% which is higher than the CFR for the same period in 2025 (18.7%).

Did you find this report useful? Have suggestions? Please leave your feedback here:

https://kdheks.co1.qualtrics.com/jfe/form/SV_9Ks1CzljzLKTOOG

Appendix A: Priority Categories

Each week, infectious disease alerts are classified into High, Medium, and Low priority categories. In the case that an alert could reasonably fall into multiple categories, the higher alert category is selected.

High Priority:

1) Diseases spread person-to-person with a high probability of transmission during mass gathering events
    a. Respiratory: MERS
    b. STIs: Mpox Clade I and II
2) High Consequence Infectious Diseases presenting an immediate threat, pose a high risk of death, or serious long-term disability
    a. VHF: Ebola, Lassa, Nipah, etc.
3) Diseases with no effective vaccine and no effective treatment
4) Potential for bioterrorism (anthrax, botulism, plague, smallpox, tularemia)
5) Newly identified infectious diseases with potential for human-to-human transmission
6) Diseases prompting a US Centers for Disease Control and Prevention Travel Health Notice of Level 4: Avoid All Travel

Medium Priority:

1) Endemic diseases within the United States with higher-than-normal activity
    a. Respiratory: RSV, Influenza, COVID-19, Legionnaire's, Tuberculosis
    b. STIs: Gonorrhea, Syphilis
    c. Foodborne/Waterborne: Hepatitis A, Listeria, *Salmonella*
    d. Vector-borne Diseases: Dengue, West Nile Virus, Rocky Mountain Spotted Fever, Ehrlichiosis, Anaplasmosis
2) Endemic diseases outside of the United States with higher-than-normal activity
    a. Respiratory: RSV, Influenza, COVID-19, Tuberculosis
    b. STIs: Gonorrhea, Syphilis
    c. Foodborne/Waterborne: Cholera, Typhoid, Hepatitis A, Giardia, Norovirus, Leptospirosis, Listeria, Legionellosis
    d. Vector-borne Diseases: Malaria, Dengue, Chikungunya, Oropouche, Japanese Encephalitis, Chagas, African Tick Bite Fever, West Nile Virus, Leishmaniasis, Rift Valley Fever, Hantavirus, Rocky Mountain Spotted Fever
3) All outbreaks with potential to affect the general population within the United States
4) All outbreaks with potential to affect the general population outside of the United States
5) Domestic and international measles outbreaks
6) Diseases prompting a US Centers for Disease Control and Prevention Travel Health Notice of Level 3: Reconsider Nonessential Travel

Low Priority:

1) New World Screwworm infestation in Mexico

2) Vaccine Preventable Disease outbreaks other than measles (mumps, rubella, pertussis, meningococcal, varicella, poliomyelitis)
3) Specific populations at an increased risk, but low risk to the general population
    a. Infant botulism
    b. Human Highly Pathogenic Avian Influenza (HPAI) case or HPAI in a Kansas flock or dairy operation
4) Newly identified infectious diseases with no potential for human-to-human transmission
5) Diseases prompting a US Centers for Disease Control and Prevention Travel Health Notice of Level 2: Practice Enhanced Precautions

Appendix B: Data Sources

BlueDot. [cited March 17, 2026]. "BlueDot Event Alerts," March 2026. Available from: https://bluedot.global/.

Boston University Center on Emerging Infectious Diseases, Biothreats Emergence, Analysis and Communications Network (BEACON) (https://beaconbio.org/en/)

Center for Infectious Disease Research and Policy (CIDRAP), Daily News feed (https://www.cidrap.umn.edu/).

European Centre for Disease Prevention and Control, Weekly Bulletin Communicable Disease Threats Report (https://www.ecdc.europa.eu/en).

Georgia Department of Public Health, Travel Clinical Assistant (CTA) tool (https://dph.georgia.gov/TravelClinicalAssistant).

New York State Department of Health, Global Health Update 3/12/2026 (https://globalhealthreports.health.ny.gov/).

World Health Organization, Integrated influenza and other respiratory viruses surveillance output (https://app.powerbi.com/view?r=eyJrIjoiNzdjZTVmY2YtNzY2NC00NTM0LTkzY2QtMWM0MzY0Mjg0YTZjIiwidCI6ImY2MTBjMGI3LWJkMjQtNGIzOS04MTBiLTNkYzI4MGFmYjU5MCIsImMiOjh9)

1) Endemic diseases outside of the United States with higher-than-normal activity
    a. RSV: RSV systematic surveillance map, Percent positive
        i. Countries with >30% positivity
    b. Influenza: Influenza systematic surveillance map, Percent positive
        i. Countries with >30% positivity
    c. COVID-19: SARS-CoV-2 systematic surveillance, Percent positive
        i. Countries with >30% positivity

World Health Organization, Global Mpox Trends (https://worldhealthorg.shinyapps.io/mpx_global/_w_2cf014b5d64c494bacff7fea3dc86bf3/_w_940e365b46e44b7ebfc21c08b8a884c2/#sec-global)

Appendix C: Map of 2026 FIFA World Cup Countries

WC host and qualifying countries: Algeria, Argentina, Australia, Austria, Belgium, Brazil, Canada, Cabo Verde (Cape Verde), Colombia, Croatia, Curacao, Ecuador, Egypt, England, France, Germany, Ghana, Haiti, Iran, Ivory Coast (Côte d'Ivoire), Japan, Jordan, Mexico, Morocco, Netherlands, New Zealand, Norway, Panama, Paraguay, Portugal, Qatar, Saudi Arabia, Scotland, Senegal, South Africa, South Korea, Spain, Switzerland, Tunisia, United States, Uruguay, Uzbekistan

Available at: https://www.kdhe.ks.gov/1508/Travel-Health under Map of 2026 FIFA World Cup Countries